SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Pembrolizumab 400 mg · colorectal

First-line metastatic MSI-H / dMMR colorectal cancer · NCCP 00558 · version 13 · 42-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Pembrolizumab aims to control bowel cancer that has spread and has a particular DNA-repair feature, called MSI-H or dMMR. Your team discusses likely benefit, uncertainty and alternatives for you.

Schedule
Pembrolizumab 400 mg is given over about 30 minutes on Day 1 every 6 weeks (42 days). The visit takes longer for assessment and preparation.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pembrolizumab
IMMUNOTHERAPY

- ☐ Immune inflammation of lungs, bowel, liver, kidneys, hormone glands, skin, heart and nervous system
- ☐ Infusion reactions
- ☐ Some effects need steroids, hospital care or long-term hormone replacement
- ☐ Effects may start after the final dose
- ☐ Heart, muscle and nerve immune effects can occur together and require urgent assessment
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Discuss the first-line MSI-H/dMMR colorectal indication, benefit and uncertainty, alternatives, immune inflammation in multiple organs, possible severe or lasting effects, infusion reactions, steroids or long-term hormone replacement, pregnancy precautions and the option to reconsider. Confirm the urgent contact plan. Discuss the possibility that heart, muscle and nerve inflammation can occur together and may need urgent hospital treatment. A severe immune reaction may mean treatment must remain stopped even after recovery. Discuss the possibility that heart, muscle and nerve inflammation can occur together and may need urgent hospital treatment. A severe immune reaction may mean treatment must remain stopped even after recovery.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00558 v13.

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